



PO No :PO1587309237-725



Name	: Mr.AMARJEET SINGH	Client Name	: TATA 1MG DEHRADUN
Age/Gender	: 70/Male	Registration Date	: 11/Aug/2024 10:04AM
Patient ID	: DDN97836	Collection Date	: 11/Aug/2024 07:48AM
Barcode ID/Order ID	: D12379242 / 10319272	Sample Receive Date	: 11/Aug/2024 10:16AM
Referred By	: Dr.	Report Status	: Final Report
Sample Type	: Whole Blood-EDTA	Report Date	: 11/Aug/2024 01:21PM

HAEMATOLOGY

FULL BODY HEALTH CHECKUP

Test Name	Result	Unit	Bio. Ref. Interval	Method
Complete Blood Count				
Hemoglobin	15.8	g/dL	13.0-17.0	Spectrophotometry
RBC	4.75	10 ⁶ /cu.mm	4.5 - 5.5	Impedance Variation
HCT	46.2	%	40 - 50	Pulse Height Average
MCV	97.3	fL	83 - 101	Calculated
MCH	33.2	pg	27 - 32	Calculated
MCHC	34.1	g/dL	31.5 - 34.5	Calculated
RDW-CV*	13.3	%	11.6-14	Calculated
Total Leucocyte Count	5.77	10 ³ /μL	4 - 10	Impedance Variation
Differential Leucocyte Count				
Neutrophils	50.7	%	40-80	DHSS/Microscopy
Lymphocytes	35.9	%	20-40	DHSS/Microscopy
Monocytes	5.4	%	2-10	DHSS/Microscopy
Eosinophils	7.1	%	1-6	DHSS/Microscopy
Basophils	0.9	%	0-2	DHSS/Microscopy
Absolute Leucocyte Count				
Absolute Neutrophil Count*	2.93	10 ³ /μL	2-7	Calculated
Absolute Lymphocyte Count*	2.07	10 ³ /μL	1-3	Calculated
Absolute Monocyte Count*	0.31	10 ³ /μL	0.2-1	Calculated
Absolute Eosinophil Count*	0.41	10 ³ /μL	0.02-0.5	Calculated
Absolute Basophil Count*	0.05	10 ³ /μL	0.02-0.1	Calculated
Platelet Count	221	10 ³ /μL	150 - 410	Impedance Variation/Microscopy
MPV*	9.9	fL	6.5 - 12	Calculated
PDW*	16.7	fL	9 - 17	Calculated

Comment:

As per the recommendation of International council for Standardization in Hematology, the differential leucocyte counts are additionally being reported as absolute numbers of each cell in per unit volume of blood.
DHSS : Double Hydrodynamic Sequential System

NABL certificate and scope



This test has been performed at
TATA 1MG DEHRADUN
Address: 2nd Floor, Plot No. 1072, Ashirwad Tower, Ballupur Road, Chakrata Rd, Sunder Vihar, Dehradun, Uttarakhand 248001

[Signature]

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Sample Type	: EDTA	Report Date	: 11/Aug/2024 01:55PM

HAEMATOLOGY

FULL BODY HEALTH CHECKUP

Test Name	Result	Unit	Bio. Ref. Interval	Method
Erythrocyte Sedimentation Rate				
Erythrocyte Sedimentation Rate	3	mm/hr	0-14	Modified westergren

Comment:

- ESR provides an index of progress of the disease and is widely used as an indicator of inflammation, infection, trauma, or malignant diseases. Changes are more significant than a single abnormal test
- It is specifically indicated to monitor the course or response to the treatment of diseases like rheumatoid arthritis, tuberculosis bacterial endocarditis ,acute rheumatic fever ,Hodgkins disease,temporal arthritis , and systemic lupus erythematosus; and to diagnose and monitor giant cell arteritis and polymyalgia rheumatica.
- An elevated ESR may also be associated with many other conditions, including autoimmune disease, anemia, infection,malignancy,pregnancy, multiple myeloma, menstruation, and hypothyroidism.
- Although a normal ESR cannot be taken to exclude the presence of organic disease, its rate is dependent on various physiologic and pathologic factors.
- The most important component influencing ESR is the composition of plasma. High level of C-Reactive Protein, fibrinogen, haptoglobin, alpha-1antitrypsin, ceruloplasmin and immunoglobulins causes the elevation of Erythrocyte Sedimentation Rate.
- Drugs that may cause increase ESR levels include: dextran, methyldopa, oral contraceptives, penicillamine, procainamide, theophylline, and Vitamin A. Drugs that may cause decrease levels include: aspirin, cortisone, and quinine

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HAEMATOLOGY

FULL BODY HEALTH CHECKUP

Test Name	Result	Unit	Bio. Ref. Interval	Method
Glycosylated Hemoglobin (HbA1c)	7.8	%	4-5.6	HPLC (NGSP certified)
Estimated average glucose (eAG)*	177.16	mg/dL		Calculated

Comment:

Interpretation: HbA1c%

≤5.6	Normal
5.7-6.4	At Risk For Diabetes
≥6.5	Diabetes

Adapted from American Diabetes Association.

Comments:

A 3 to 6 monthly monitoring is recommended in diabetics. People with diabetes should get the test done more often if their blood sugar stays too high or if their healthcare provider makes any change in the treatment plan. HbA1c concentration represent the integrated values for blood glucose over the preceding 8-12 weeks and is not affected by daily glucose fluctuation, exercise & recent food intake.

Please note, Glycemic goal should be individualized based on duration of diabetes, age/life expectancy, comorbid conditions, known CVD or advanced microvascular complications, hypoglycemia unawareness, and individual patient considerations.

Factors that interfere with HbA1c Measurement: Hemoglobin variants, elevated fetal hemoglobin (HbF) and chemically modified derivatives of hemoglobin (e.g. carbamylated Hb in patients with renal failure) can affect the accuracy of HbA1c measurements.

Factors that affect interpretation of HbA1c Measurement: Any condition that shortens erythrocyte survival or decrease mean erythrocyte age (e. g., recovery from acute blood loss, hemolytic anemia, HbSS, HbCC, and HbSC) will falsely lower HbA1c test results regardless of the assay method used. Iron deficiency anemia is associated with higher HbA1c.

Note: Presence of Hemoglobin variants and/or conditions that affect red cell turnover must be considered, particularly when the HbA1c result does not correlate with the patient's blood glucose levels.

- HPLC - High performance liquid chromatography

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Sample Type	: Serum	Report Date	: 11/Aug/2024 01:55PM

BIOCHEMISTRY

FULL BODY HEALTH CHECKUP

Test Name	Result	Unit	Bio. Ref. Interval	Method
Lipid Profile				
Cholesterol - Total	89	mg/dL	Desirable <200, Borderline High 200-239, High >=240	Enzymatic
Triglycerides	136	mg/dL	Normal: < 150, Borderline: 150 - 199, High:200 - 499, Very High >=500	GPO, Trinder without serum blank
Cholesterol - HDL	34	mg/dL	Undesirable/high risk <=40mg/dL Desirable/low risk>=60mg/dl	Elimination/catalase
Cholesterol - LDL*	28	mg/dL	Desirable:<100 Above desirable:100-129 Borderline high :130-159 High :160-189 Very high :>=190	Calculated
Cholesterol- VLDL*	27	mg/dL	<30	Calculated
Cholesterol : HDL Cholesterol*	2.7	Ratio	Desirable : 3.5-4.5 High Risk : >5	Calculated
LDL : HDL Cholesterol*	0.84	Ratio	Desirable : 2.5-3.0 High risk : >3.5	Calculated
Non HDL Cholesterol*	55	mg/dL	Desirable:< 130, Above Desirable:130 - 159, Borderline High:160 - 189, High:190 - 219, Very High: >= 220	Calculated

Comment:

- Lipid profile measurements in the same patient can show physiological & analytical variations. It is recommended that 3 serial

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BIOCHEMISTRY

FULL BODY HEALTH CHECKUP

Test Name	Result	Unit	Bio. Ref. Interval	Method
samples 1 week apart may be tested.				
●Indians are at a high risk of developing atherosclerotic cardiovascular disease (ASCVD); at a much earlier age and more severe with high mortality. Dyslipidemia (abnormal lipid profile) is the major risk factor and found in almost 80% Indians.				
●Total cholesterol is the total amount of cholesterol in blood comprising of HDL, LDL-C, and VLDL.				
●LDL Cholesterol (LDL-C) or "bad"cholesterol contributes most significantly to atherosclerosis leading to heart disease or stroke and is the primary target for reducing risk for cardiovascular disease.				
●High-density lipoprotein (HDL) or "good" cholesterol can lower risk of heart disease and stroke.				
●Triglyceride (TG) level also plays a major role in CVD. Indians are more prone to Atherogenic dyslipidemia, a condition associated with high TG, low HDL-C and high LDL-C; this is associated with diabetes, metabolic syndrome and insulin resistance. Hence high triglyceride levels also need to be treated.				
●Non-HDL-Cholesterol (Non-HDLC) measures all plaque forming lipoproteins (e.g. remnants, LDL-C, VLDL, Lp(a), Apo-B). Monitoring of Non-HDLC is important in patients with high TG (e.g. diabetics, obese persons) and those already on statin therapy.				
●Lipid Association of India (LAI-2020) recommends:-				
● Screening of all Indians above the age of 20 years for CVD risk factors, esp. lipid profile.				
● Identification of Risk factors: Age (male ≥45 years, female ≥55 years); Family h/o heart disease at younger age (<55 yrs in males, <65 yrs in female), Smoking/tobacco use, High blood pressure, Low HDL (males <40 mg/dl and females <50mg/dl).				
● Fasting lipid profile is not mandatory for screening. Both fasting and non-fasting lipid profiles are equally important for managing Indian patients.				
● Non-HDLC should be calculated in every subject. LAI recommends LDL-C as the primary target and Non-HDLC as the co-primary target for initiating drug therapy.				
● Lifestyle modifications are of first and foremost importance for management and prevention of dyslipidemia. Among low risk groups, treatment is started only after 3 months of lifestyle changes.				
● Testing for Apolipoprotein B, hsCRP, Lp(a) should be considered for patients in moderate risk group.				
● Newer treatment goals based on Risk Groups and values of LDL-C and Non-HDLC				

New treatment goals by Lipid Association of India (2020)

	CONSIDER THERAPY (cut-off level)		TREATMENT GOALS	
Risk groups	LDL-C (mg/dL)	Non-HDLC (mg/dL)	LDL-C (mg/dL)	Non-HDLC (mg/dL)
Extreme Risk Gp Cat. A	≥50	≥80	<50 (Optional ≤30)	<80 (Optional ≤60)
Extreme Risk Gp Cat. B	>30	>60	≤30	≤60
Very High Risk	≥50	≥80	<50	<80
High Risk	≥70	≥100	<70	<100
Moderate Risk	≥100	≥130	<100	<130
Low risk	≥130*	≥160*	<100	<130

*After an adequate non-pharmacological intervention for at least 3 months

●As per NCEP Expert Panel (2011) guidelines, universal screening for dyslipidemia is recommended for children between 9 - 11 yrs (repeat at 17-21 yrs). Screening is not recommended before the age of 2yrs. Above the age of 2 yrs, selective screening is done in children with family history of premature CVD or risk factors like obesity, diabetes, and hypertension.

Note: Reference Interval as per National Cholesterol Education Program (NCEP) Report.

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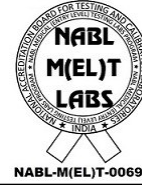

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BIOCHEMISTRY

FULL BODY HEALTH CHECKUP

Test Name	Result	Unit	Bio. Ref. Interval	Method
LIVER FUNCTION TEST				
Liver Function Test				
Bilirubin-Total	0.49	mg/dL	0.2 - 1.1	Vanadate oxidation
Bilirubin-Direct*	0.23	mg/dL	0.0-0.3	Vanadate oxidation
Bilirubin-Indirect*	0.26	mg/dL	0.2-0.8	Calculated
Protein, Total	7.48	g/dL	5.7-8.2	Biuret
Albumin	4.46	g/dL	3.2-4.8	BCG Dye Binding
Globulin*	3.0	g/dl	2.1 - 3.9	Calculated
A/G Ratio*	1.48	Ratio	0.8 - 2.1	Calculated
Aspartate Transaminase (SGOT)	25	U/L	<34 U/L	Modified IFCC
Alanine Transaminase (SGPT)	37	U/L	10-49	Modified IFCC
SGOT/SGPT*	0.68	Ratio		Calculated
Alkaline Phosphatase*	57	U/L	46-116	IFCC Standardization
Gamma Glutamyltransferase (GGT)*	16	U/L	<73	Modified IFCC

Comment:

- Raised ALT and AST indicate hepatocellular damage (e.g. viral or drugs etc). ALT is more liver-specific while AST is also found in heart, skeletal muscle, and kidney. Mild elevation (less than twice normal) often resolves on its own. Fatty liver disease (especially with metabolic syndrome) is a common cause in asymptomatic cases. Certain drugs (paracetamol, statins), herbal supplements, energy drinks, and antibiotics may also affect liver function.
- SGOT/SGPT Ratio: Typically <1 in healthy individuals (vary between 0.7-1.4; higher in women than men). High SGPT (ratio <1) seen in acute or chronic hepatitis, autoimmune disorders, medications, toxins while ratio >1 indicates alcoholic hepatitis, cirrhosis, metastasis or non-hepatic issues (hemolytic diseases, CVS disorders).
- Elevated Alkaline Phosphatase and GGT: Suggest cholestatic diseases (e.g. bile duct obstruction, primary biliary cirrhosis etc.) and can also be due to bone disease, pregnancy, chronic renal failure, malignancy, and congestive heart failure.
- High Bilirubin: Indicates jaundice due to increased RBC breakdown, liver damage (e.g., infections, toxins), or cholestasis (e.g., gallstones, tumors).
- High Protein Levels: Seen in dehydration (e.g., severe vomiting, diarrhea) or increased production (e.g., inflammation,

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BIOCHEMISTRY

FULL BODY HEALTH CHECKUP

Test Name	Result	Unit	Bio. Ref. Interval	Method
hematopoietic neoplasms). Low protein and albumin: Result from impaired synthesis (liver disease), decreased intake, tissue damage, malabsorption, or increased renal excretion.				

Kidney Function Test.

Blood Urea Nitrogen*	6	mg/dL	9.0-23.0	Urease with GLDH
Urea*	12.33	mg/dL	19.26 - 49.22	Calculated
Creatinine	0.84	mg/dL	0.7-1.3	Alkaline picrate-kinetic
Uric Acid	5.9	mg/dL	3.7-7.2	Uricase/Peroxidase
Sodium	135	mmol/L	132-146	Direct ISE
Potassium	4.30	mmol/L	3.5-5.3	Direct ISE
Chloride	108.0	mmol/L	99-109	Direct ISE
BUN/Creatinine Ratio*	6.9	Ratio	12:1 - 20:1	Calculated

Comment:

BUN is directly related to protein intake and nitrogen metabolism and inversely related to the rate of excretion of urea. Blood urea nitrogen (BUN) levels reflect the balance between the production and excretion of urea. Increased levels are seen in renal failure (acute or chronic), urinary tract obstruction, dehydration, shock, burns, CHF, GI bleeding, nephrotoxic drugs. Decreased levels are seen in hepatic failure, nephrotic syndrome, cachexia (low-protein and high-carbohydrate diets).

Urea is a non-proteinous nitrogen compound formed in the liver from ammonia as an end product of protein metabolism. Urea diffuses freely into extracellular and intracellular fluid and is ultimately excreted by the kidneys. Increased levels are found in acute renal failure, chronic glomerulonephritis, congestive heart failure, decreased renal perfusion, diabetes, excessive protein ingestion, gastrointestinal (GI) bleeding, hyperalimentation, hypovolemia, ketoacidosis, muscle wasting from starvation, neoplasms, pyelonephritis, shock, urinary tract obstruction, nephrotoxic drugs. Decreased levels are seen in inadequate dietary protein, low-protein/high-carbohydrate diet, malabsorption syndromes, pregnancy, severe liver disease, certain drugs.

Creatinine is catabolic product of creatinine phosphate, which is excreted by filtration through the glomerulus and by tubular secretion. Creatinine clearance is an acceptable clinical measure of glomerular filtration rate (GFR). Increased levels are seen in acute/chronic renal failure, urinary tract obstruction, hypothyroidism, nephrotoxic drugs, shock, dehydration, congestive heart failure, diabetes. Decreased levels are found in muscular dystrophy.

BUN/Creatinine ratio (normally 12:1-20:1) is decreased in acute tubular necrosis, advanced liver disease, low protein intake, and following hemodialysis. BUN/Creatinine ratio is increased in dehydration, GI bleeding, and increased catabolism.

Uric acid levels show diurnal variation. The level is usually higher in the morning and lower in the evening. Increased levels are seen in starvation, strenuous exercise, malnutrition, or lead poisoning, gout, renal disorders, increased breakdown of body cells in some cancers (including leukemia, lymphoma, and multiple myeloma) or cancer treatments, hemolytic anemia, sickle cell anemia, or heart failure, pre-eclampsia, liver disease (cirrhosis), obesity, psoriasis, hypothyroidism, low blood levels of parathyroid hormone (PTH), certain drugs, foods that are very high in purines - such as organ meats, red meats, some seafood and beer. Decreased levels are seen in liver disease, Wilson's disease, Syndrome of inappropriate antidiuretic hormone (SIADH), certain drugs.

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BIOCHEMISTRY

FULL BODY HEALTH CHECKUP

Test Name	Result	Unit	Bio. Ref. Interval	Method
Iron Studies, Basic				
Iron Serum	87	µg/dL	65-175	Ferrozine
Unsaturated Iron Binding Capacity	191	µg/dL	111 - 343	Ferrozine
Total Iron Binding Capacity (TIBC)	277.87	µg/dL	240-450	Calculated
Transferrin Saturation	31.31	%	16 - 50	Calculated

Comment:

Iron is an essential trace mineral element which forms an important component of hemoglobin, metallocompounds and Vitamin A. Deficiency of iron is seen in iron deficiency and anaemia of chronic disorders. Increased iron concentration are seen in hemolytic anaemias, hemochromatosis and acute liver disease. Serum Iron alone is unreliable due to considerable physiologic diurnal variation in the results with highest values in the morning and lowest values in the evening as well as variation in response to iron therapy .

Total Iron Binding capacity (TIBC) is a direct measure of the protein Transferrin which transports iron from the gut to storage sites in the bone marrow. Increased levels of TIBC suggest that total iron body stores are low, increased concentration may be the sign of Iron deficiency anaemia, polycythemia vera ,and may occur during the third trimester of pregnancy. Decreased levels may be seen in hemolytic anaemia, hemochromatosis, chronic liver disease, hypoproteinemia ,malnutrition.

Unsaturated Iron Binding Capacity (UIBC) is increased in low iron state and decreased in high iron concentration such as hemochromatosis. In case of anaemia of chronic disease the patient may be anaemic but has adequate iron reserve and a low uIBC.

Transferrin Saturation occurs in Idiopathic hemochromatosis and Transfusional hemosiderosis where no unsaturated iron binding capacity is available for iron mobilization. Similar condition is seen in congenital deficiency of Transferrin.

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Immunology

FULL BODY HEALTH CHECKUP

Test Name	Result	Unit	Bio. Ref. Interval	Method
Thyroid Profile				
T3, Total	0.94	ng/mL	0.60-1.81	CLIA
T4, Total	8.1	µg/dl	4.5-12.6	CLIA
Thyroid Stimulating Hormone - Ultra Sensitive	5.560	uIU/ml	0.55-4.78	CLIA

Comment:

- Below mentioned are the guidelines for pregnancy related reference ranges for TSH, total T3 & Total T4.

Pregnancy			
	TSH (µIU/mL) (as per American Thyroid Association)	Total T3 (ng/mL)	Total T4(µg/dL)
1st trimester	0.1-2.5	0.81-1.90	7.33-14.8
2nd trimester	0.2-3.0	1.00-2.60	7.93-16.1
3rd trimester	0.3-3.0	1.00-2.60	6.95-15.7

- TSH levels are subject to circadian variation, reaching peak levels between 2 - 4.a.m. and at a minimum between 6-10 pm .
- The variation is of the order of 50%, hence time of the day has influence on the measured serum TSH concentrations.
- TSH is secreted in a dual fashion: Intermittent pulses constitute 60-70% of total amount, background continuous secretion is 30-40%.These pulses occur regularly every 1-3 hrs.
- Total T3 & T4 concentrations are altered by physiological or pathological changes in thyroxine binding globulin (TBG) capacity .
- The determination of free T3 & free T4 has the advantage of being independent of changes in the concentrations and binding properties of the binding proteins.
- Changes in thyroid status are typically associated with concordant changes in T3, T4 and TSH levels.
- Unexpectedly abnormal or discordant thyroid test values may be seen with some rare, but clinically significant conditions such as central hypothyroidism, TSH-secreting pituitary tumors, thyroid hormone resistance, or the presence of heterophilic antibodies (HAMA) or thyroid hormone autoantibodies.
- For diagnostic purposes, results should be used in conjunction with other data.

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Immunology

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Test Name		Result	Unit	Bio. Ref. Interval	Method
TSH	T3	T4	Interpretation		
High	Normal	Normal	Subclinical Hypothyroidism		
Low	Normal	Normal	Subclinical Hyperthyroidism		
High	High	High	Secondary Hyperthyroidism		
Low	High/Normal	High/Normal	Hyperthyroidism		
Low	Low	Low	Non thyroidal illness / Secondary Hypothyroidism		



NABL certificate
and scope



This test has been performed at
TATA 1MG DEHRADUN
Address: 2nd Floor, Plot No. 1072, Ashirwad
Tower, Ballupur Road, Chakrata Rd, Sunder
Vihar, Dehradun, Uttarakhand 248001

Dr. Amit Goyal
MBBS, MD (Pathology)
Consultant Pathologist
Reg No. 22491

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PO No :PO1587309237-725



Name	: Mr.AMARJEET SINGH	Client Name	: TATA 1MG DEHRADUN
Age/Gender	: 70/Male	Registration Date	: 11/Aug/2024 10:04AM
Patient ID	: DDN97836	Collection Date	: 11/Aug/2024 07:48AM
Barcode ID/Order ID	: D12379244 / 10319272	Sample Receive Date	: 11/Aug/2024 10:23AM
Referred By	: Dr.	Report Status	: Final Report
Sample Type	: Urine	Report Date	: 11/Aug/2024 01:21PM

CLINICAL PATHOLOGY

FULL BODY HEALTH CHECKUP

Test Name	Result	Unit	Bio. Ref. Interval	Method
Urine Routine & Microscopy				
Colour*	Yellow		Pale Yellow	Manual
Appearance*	Clear		Clear	Manual
Specific gravity	1.010		1.003 - 1.035	pKa change
pH	5.5		4.6 - 8.0	Double Indicator
Glucose	Negative		Negative	GOD-POD
Protein	Negative		Negative	Protein Error Principle
Ketones	Negative		Negative	Nitroprusside
Blood	Negative		Negative	Peroxidase
Bilirubin	Negative		Negative	Diazonium
Urobilinogen	Normal		Normal	Ehrlich
Leucocyte Esterase	Negative		Negative	Pyrrole
Nitrite	Negative		Negative	P-arsanilic acid
Pus cells*	1-2	/hpf	0-5	Microscopy
Red Blood Cells*	Nil	/hpf	0-2	Microscopy
Epithelial cells*	0-1	/hpf	Few	Microscopy
Casts*	Nil	/lpf	Nil	Microscopy
Crystals*	Nil		Nil	Microscopy
Yeast*	Nil		Nil	Microscopy
Bacteria*	Nil		Nil	Microscopy

Comment:

•Note: Pre-test condition to be observed while submitting the sample-first void, mid stream urine, collected in a clean, dry, sterile container is recommended for routine urine analysis, avoid contamination with any discharge from vaginal, urethra, perineum, Avoid prolonged transit time & undue exposure to sunlight.

•During interpretation, points to be considered are Negative nitrite test does not exclude the urinary tract infections. Trace proteinuria can be seen with many physiological conditions like prolonged recumbency, exercise, high protein diet. False positive reactions for bile pigments, proteins, glucose and nitrites can be caused by peroxidase like activity by disinfectants, therapeutic dyes, ascorbic acid and certain drugs. • Urine microscopy is done in centrifuged urine specimens

*** End Of Report ***

NABL certificate
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* The parameters marked with an * are not- recognized by NABL.

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PO No :PO1587309237-725



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CLINICAL PATHOLOGY

FULL BODY HEALTH CHECKUP

Test Name	Result	Unit	Bio. Ref. Interval	Method
Conditions of Laboratory Testing & Reporting: Test results released pertain to the sample, as received. Laboratory investigations are only a tool to facilitate in arriving at a diagnosis and should be clinically correlated by the interpreting clinician. Result delays may happen because of unforeseen or uncontrollable circumstances. Test report may vary depending on the assay method used. Test results may show inter-laboratory variations. Test results are not valid for medico-legal purposes. Please mail your queries related to test results to Customer Care mail ID care@1mg.com				
Disclaimer: Results relate only to the sample received. Test results marked "BOLD" indicate abnormal results i.e. higher or lower than normal. All lab test results are subject to clinical interpretation by a qualified medical professional. This report cannot be used for any medico-legal purposes. Partial reproduction of the test results is not permitted. Also, TATA 1mg Labs is not responsible for any misinterpretation or misuse of the information. The test reports alone may not be conclusive of the disease/condition, hence clinical correlation is necessary. Reports should be vetted by a qualified doctor only.				

TATA 1mg | Labs

NABL certificate
and scope

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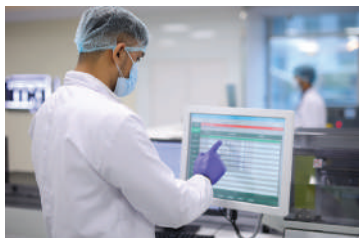
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